Enacting AI Safety in Relational Practice: Scaffolding Practical Judgment in Peer Support

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Abstract

AI is increasingly entering relational practice, where workers support people through interpersonal relationships. General AI safety guidance cannot fully specify safe action because what safety requires depends on particular people and relationships and can change as relationships unfold. Through interviews with 19 peer specialists, we investigate how they reason about navigating AI safety and their support needs. Our findings identify what support for AI safety should enable, when and how it should scaffold practical judgment, and conditions for sustaining and evolving such support. We conceptualize support for AI safety in relational practice as scaffolding the enactment of AI safety: helping workers interpret safety principles for particular people and relationships, translate those interpretations into action, and revisit earlier judgments as they learn how AI's involvement affects people and relationships over time. We derive design implications for scaffolding practical judgment at relational moments and using emerging safety-relevant knowledge to continually evolve support.

CCS Concepts

• **Human-centered computing → Empirical studies in HCI**.

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1 Introduction

Artificial intelligence (AI)—particularly large language models (LLMs)—is increasingly integrated into the work of frontline workers in high-stakes domains such as mental health [15, 47, 75, 81, 94] and social services [48, 101]. Workers' decisions and interactions can have significant consequences for people's well-being, access to support, and trust in care. In many of these settings, interpersonal relationships are central to how workers understand and support the people they serve. These relationship-centered ways of working have been described as *"relational practice"*, in which understanding, judgment, and action are shaped by the particular people, relationships, and contexts in which interactions unfold [25, 30, 40, 51].

Peer support is a salient example of a high-stakes relational practice. Peer specialists, also referred to as peer support workers [67, 95], support service users navigating significant mental health conditions alongside substance use, housing instability, employment difficulties, criminal justice involvement, and other socioeconomic challenges [37, 41, 62]. Many service users also face compounding experiences of marginalization and exclusion that can make sustained engagement with traditional systems of care difficult. In 2024 alone, an estimated 4.1 million adults in the United States received services from a peer specialist, including 1.2 million adults with serious mental illness (SMI), such as schizophrenia, bipolar disorder, and major depressive disorder [96]. Drawing on their own lived experiences, peer specialists provide emotional support, practical knowledge, and resources while helping service users navigate everyday challenges [73, 97, 100, 109]. Peer support is non-clinical and non-directive, grounded in values such as

mutuality, self-determination, and person-centered relationships [73, 74, 91].

Emerging uses of AI and LLMs in relational practices such as peer support and behavioral health care have shown the potential to support empathic communication, broaden the information and resources frontline workers can bring into interactions, and tailor support to individual needs and circumstances [49, 74, 88, 98]. Unlike conventional information-retrieval technologies, LLMs do not simply retrieve existing information but generate responses adapted to the context provided about a service user and situation. These responses can appear fluent and authoritative even when they are inaccurate, uncertain, or poorly suited to the particular person or situation. We use ***AI safety*** broadly to refer to preventing and mitigating harms that may arise from AI use, including harms arising from AI outputs and from how AI is incorporated into interactions and relationships [5, 89]. These generative capabilities introduce distinct safety concerns in relational settings because harms can arise not only from what the system produces, but also from how its outputs are interpreted, communicated, and incorporated into ongoing interactions. An inaccurate or decontextualized response, for example, may not only provide poor information, but also undermine trust or fail to reflect a service user's circumstances and lived experiences. Even when AI outputs are not overtly harmful, how AI is introduced and used within an interaction can make support feel less authentic, weakening the human connection on which relational practice depends [74, 90, 98]. Together, these concerns suggest that AI safety in relational practice must account for impacts on both the people being supported and the very *relationships* through which that support is provided.

These growing concerns have prompted the HCI community to develop tools, resources and guidance that help frontline workers identify and mitigate AI-related harms during system development [27, 63, 82, 113], deliberate on sociotechnical risks during adoption and deployment [44, 84, 101], and contextualize safe and responsible-AI principles within organizational practice [56]. This work demonstrates that AI safety cannot be reduced to technical properties and must be interpreted within particular organizational and social contexts. However, less is known about how workers navigate safety in ongoing relational practice, where the context relevant to safety can itself emerge and change through interaction. Frontline worker-facing guidance in behavioral health offers important recommendations around consent, privacy, bias, and practical judgment for relational practice [2, 3, 33, 47, 78, 83, 102, 114], yet high-level recommendations cannot fully specify what safe action looks like in a particular relational encounter. We use *practical judgment* to refer to determining an appropriate course of action by interpreting general knowledge, values, and principles in light of the particulars of a situation [31, 71]. We know less about how peer specialists exercise this judgment to navigate AI safety within their relationships with service users and what support can scaffold this process.

To address the gap, we examine AI safety in peer support as a case of high-stakes relational practice. We ask: ***How do peer specialists reason about navigating AI safety in their relational practice, and what support do they need to do so?***

To answer this question, we recruited 19 peer specialists in the U.S. and conducted semi-structured interviews examining how they reasoned about and responded to AI-related safety concerns in peer-support practice. During the interviews, we used PeerAlly, an LLM-based research prototype designed around common peer support tasks identified in prior work [74]: translating broad goals into concrete next steps and locating relevant programs, benefits, and community resources. This case provided a concrete and practice-relevant context for investigating AI safety in peer-support practice. Our study did not presume that AI should be incorporated into peer support; whether, when, and how AI should be involved was itself treated as a safety question open to participants' judgment. We used two complementary materials to scaffold the interview. First, participants gained hands-on experience with PeerAlly, grounding their reasoning about AI safety in relation to a system that could plausibly become part of their everyday work. Second, building on risks identified in prior studies of AI in peer support [74, 98], we developed five AI risk scenario cards that participants used to discuss potential safety concerns and the support they would need to address them. Finally, participants reflected on how such support could be sustained and improved over time.

Our findings identified what support for AI safety should enable in relational practice, when and how it should scaffold practical judgment as relationships with service users unfold, and the organizational conditions for sustaining and evolving such support. Importantly, participants described how judgments made earlier in a relationship may need to be reconsidered in light of what they learn as that relationship unfolds. Drawing on participants' reasoning about navigating AI safety and the support they would need, we propose that support for AI safety in relational practice should scaffold the *enactment of AI safety*. We conceptualize this enactment as a recursive process in which workers (i) interpret general safety objectives in relation to a particular person and relationship, (ii) translate that interpretation into action, and (iii) revisit earlier safety judgments as they learn how AI's involvement affects people and relationships over time. This recursion is particularly consequential in relational practice because involving AI can itself affect the person and relationship, generating new knowledge that may change what safety requires in subsequent interactions. To translate this conceptualization into design, we offer two implications. First, support for AI safety should be organized and delivered around relational moments where practical judgment is required. Second, mechanisms for reflection and collective learning should help workers revisit earlier judgments, carry emerging safety-relevant knowledge into future practice, and inform the ongoing evolution of such support.

We contribute (i) empirical insights into how peer specialists reason about navigating AI safety in relational practice and the support needed to do so, and (ii) a conceptual framing of support for AI safety in relational practice as scaffolding the *enactment of AI safety*, along with design implications for supporting and sustaining this process.

2 Related Work

2.1 AI Safety in High-Stakes Relational Practice

Research in HCI and AI has extensively characterized the risks and harms that arise as AI systems are integrated into high-stakes settings. Foundational work across CHI, CSCW, and FAccT has identified concerns including hallucination, representational harm,

value misalignment, privacy leakage, and the amplification of existing social inequities [13, 89, 108], while research on deployed LLMs has documented overconfident or misleading responses and failures to handle sensitive situations appropriately [19, 36, 55]. These risks become especially consequential in relational practices, where harm can arise not only from what AI produces, but also from how AI is incorporated into the relationship [74].

Peer support provides a concrete case. In this setting, support is provided *through* an interpersonal relationship. An AI-generated response reaches a service user only after being interpreted and communicated by a peer specialist within the relationship they have built, and much of its effect is determined through this process. Peer specialists have raised concerns that AI use can leak sensitive information, or produce clinical or overly directive responses that leave service users feeling misunderstood or stigmatized [6, 74, 98]. What makes these risks consequential is not only the content of the response, but also how its introduction affects the relationship: it can weaken a service user's trust in the peer specialist, signal that they are being processed rather than heard, or displace the peer specialist's practical judgment and relational authority [74]. As a result, the same output can be safe for one service user and harmful to another, or safe at one point in a relationship and harmful at another. Safety is not fully determinable in advance of the interaction, but is partly established within it, through practical judgment about a particular person and moment.

Recent work has begun to address AI-related risks in relational practice by proposing system-level safeguards and examining frontline workers' experiences with AI. Prior work has called for incorporating peer perspectives into model auditing, aligning model behavior with peer values, and specifying peer-oriented language during model development [74, 98]. Other studies have examined how behavioral health workers encounter AI-related risks, employ AI in their work, and reason about safer use [33, 47, 102]. Collectively, this work has advanced both what can be built into systems in advance and what risks workers perceive and encounter. It has given less attention, however, to situations in which neither system-level safeguards nor general safety recommendations can fully determine what safe action requires for a particular person and relationship: moments that call for peer specialists' practical judgment and support that can scaffold that judgment.

2.2 Supporting Safe and Responsible AI Practice

A substantial body of HCI and responsible AI research has developed tools, resources, and guidance for AI practitioners working to make AI systems safer and more responsible. Much of this work has focused on those involved in developing and deploying AI, including tools and frameworks for identifying harms, addressing fairness and accountability, and integrating responsible AI considerations into organizational workflows [27, 38, 39, 56, 63, 82, 99]. For example, Madaio et al. [56] co-designed fairness checklists that fit into existing AI development processes, and Wu et al. [113] developed tools to scaffold responsible AI reasoning and collaboration within cross-functional teams.

Often, those who must use AI safely and responsibly did not build the system themselves. In response, research has examined how non-AI practitioners can be supported in understanding AI capabilities and limitations, calibrating reliance on AI recommendations, and evaluating AI outputs [20, 45, 65, 116]. In parallel, practical guidance has proliferated: governments have issued national frameworks and impact assessments [7–9], procurement and due diligence guidance [68, 107], and regulatory privacy checklists [69, 70]; vendors have published deployment documentation [61, 72]; and universities have released institutional policies for staff and students [4, 22, 103–106]. Recent work has synthesized these resources into actionable implementation practices while noting that they are not one-size-fits-all and must be tailored to particular use cases [26]. Across these efforts, support predominantly takes the form of *prescribed action*: principles to uphold, practices to follow, and steps to complete. This guidance is valuable in establishing what AI users should attend to, but because it specifies actions in advance of the situations in which they will be taken, it devotes comparatively less attention to the judgment required to determine what a given principle demands in a particular case.

This limitation becomes especially consequential for frontline relational work, where AI use is embedded in ongoing relationships with the people they support. HCI research has examined how workers in health, social services, and education encounter AI-related risks and what support they need to navigate them [28, 44, 48, 84, 101, 115]. In behavioral health, guidance for behavioral health workers addresses informed consent, privacy and confidentiality, responsibility and accountability, human oversight, and AI tool evaluation [1, 2, 78, 83, 112, 114]. Some of these resources explicitly recognize that safe use will require practical judgment adapted to particular care contexts [2].

What remains unresolved, however, is how workers engaged in relational practice exercise practical judgment when general AI safety guidance cannot fully specify in advance what safe action requires. In peer support, the same principle may require different actions across people, relationships, and moments, and relevant context may emerge only through interaction, requiring earlier decisions to be revisited as relationships evolve [35, 58–60]. This paper examines how peer specialists reason about the practical judgment involved in navigating AI safety and when and how support can scaffold that process.

3 Methods

We conducted semi-structured interviews with 19 peer specialists to examine how they understand and navigate AI safety in relational practice and what support they need to do so. To support knowledge elicitation during the interviews, we used two complementary materials: hands-on interaction with PeerAlly and AI risk scenario cards. We first describe the research context and elicitation materials, followed by participants, the elicitation procedure, and data analysis.

3.1 Context of the Research

We examine AI safety in peer support as a case of high-stakes relational practice. Peer specialists work with service users navigating a wide range of mental health, recovery, and everyday life challenges, making judgments that are shaped by individuals' circumstances, relationships, goals, and preferences. To investigate how peer specialists reason about AI-related safety concerns within

this relational practice, we used PeerAlly as a concrete AI system around which participants could ground their reflections.

As part of a broader research effort examining how AI might support peer-support practice, members of the research team developed PeerAlly, an LLM-based research prototype informed by existing research on how AI could be integrated into peer-support practice [74]. It is designed to support common tasks in peer-support practice, including translating broad goals into concrete next steps and locating relevant programs, benefits, and community resources. The system uses retrieval-augmented generation over a curated database of peer-support resources. Through its *Chat + Explore* interface, peer specialists can describe a service user's situation in their own words and iteratively refine what the system returns, receiving goals, next steps, and linked resources. PeerAlly can be used before, during, or after sessions, either by peer specialists or collaboratively with service users.

At the time of the study, PeerAlly was a research prototype rather than an operational system deployed in participants' workplaces. We therefore used it as an elicitation material that provided participants with a concrete and practice-relevant example of how an LLM-based system might intersect with peer-support practice. The study did not presume that AI should be incorporated into peer support; whether, when, and how AI should be involved remained open questions for participants to consider. This design allowed us to investigate how participants recognized and reasoned about potential AI-related risks without requiring them to have prior experience using AI as part of their formal work.

3.2 Interview Materials

We combined hands-on interaction with PeerAlly and scenario-based discussions of AI-related risks to ground participants' reasoning about AI safety in concrete peer-support practice.

3.2.1 PeerAlly Hands-on Experience. To establish a shared understanding of the system, participants received access to PeerAlly before the interview and were asked to explore its core features in advance (see Appendix A). During the interview, they returned to PeerAlly for additional hands-on exploration. This familiarization was intended to ground subsequent discussion in a concrete system that could plausibly become part of participants' everyday work, rather than in AI use in the abstract (Section 3.4).

3.2.2 AI Risk Scenario Cards. We developed a set of illustrated AI risk scenario cards to elicit how peer specialists would reason about AI safety in concrete peer-support situations. The literature on AI use in peer support describes a range of potential risks [74, 98]. Drawing on these accounts, the research team constructed hypothetical, illustrative scenarios for the present study. Rather than representing typical peer-support encounters, these scenarios were designed to make potential AI-related harms concrete and elicit participants' reasoning about how such situations might be navigated in practice. These scenarios captured AI-related risks broadly, encompassing potential harms arising from AI system behavior and from how AI is configured, interpreted, and incorporated into peer-support interactions.

Scenario-based methods are widely used in HCI to elicit lived experience, values, and context-specific knowledge of practice [42, 76, 110], scaffold reasoning about sociotechnical and ethical dilemmas [54, 111], and elicit perspectives and support ideation [48, 64, 74]. We therefore designed brief scenarios to prompt participants to draw on their practice knowledge when articulating their considerations, practical judgments, responses, and support needs [53]. We used simple visual illustrations to make the situations easier to grasp across participants with varying levels of AI and technology familiarity [23, 32, 85], given documented variation in technology literacy and experience within the peer-specialist workforce [34, 93].

To develop the cards, we first synthesized 11 initial risk concepts from the literature on AI use in peer support described above. The research team iteratively refined these concepts, consolidating overlapping categories and selecting five distinct, practice-relevant concerns: (1) Clinical and Medicalized Framing, (2) Service User Privacy and Confidentiality Breaches, (3) Hallucinated or Inaccurate Support Recommendations, (4) Loss of Peer Autonomy and Relational Authority, and (5) Context-Insensitive Support Recommendations. These concerns were presented on the scenario cards using concise, accessible language to support participants' understanding. Each category informed a one-page illustrated scenario depicting an interaction between a peer specialist, a service user, and PeerAlly. The full set of five cards is provided in Appendix B.

3.3 Participants

We recruited 19 participants by circulating study flyers within peer-support networks, supplemented by snowball sampling. Eligibility required participants to be certified peer specialists and to have basic digital literacy, including the ability to type, send messages, browse the web, create an online account, and use Zoom. Because our research question focused on supporting workers in navigating AI safety, we recruited peer specialists to examine the practical judgments they make and the support they need around AI involvement in their work. The scenarios elicited peer specialists' perspectives on the potential effects of AI use on their interactions and relationships with service users.

Participants varied in their prior experience with LLMs. All reported at least some prior use, most commonly of ChatGPT, although the depth of their experience varied. Following prior HCI work [21, 43, 77], we asked participants to self-rate their familiarity with technology and AI on separate 7-point scales (1 = not at all familiar, 7 = very familiar). Among participants who provided these ratings, scores generally fell toward the middle to upper end of the scales, with some variation across participants.

Participants ranged in age from 21 to 66; the sample was predominantly female (12 of 19 participants) and included participants identifying as White, Black, Latino, South Asian, and multiracial, with education ranging from a GED/high school diploma to a master's degree. Participants' peer-support experience ranged from 3 months to approximately 20 years. They brought experience across varied peer-support roles and organizational settings, allowing them to draw on existing practices, structures, and experiences when reasoning about how support for navigating AI safety might be integrated into peer-support work. To reduce the risk of participant identification, we report demographic characteristics only in aggregate and omit individual-level age, gender, race/ethnicity,

Participant Information	Summary
Peer-support experience	Mean: 4.4 yrs Minimum: 3 mo Maximum: 20 yrs
Technology familiarity	Mean: 5.9 Range: 5–7
AI familiarity	Mean: 5.6 Range: 2–7

Table 1: Aggregated characteristics of study participants ($N = 19$). Technology and AI familiarity were self-reported on 7-point scales (1 = not at all familiar, 7 = very familiar).

and education from Table 1. Participants were compensated $60 per session.

3.4 Interview Procedure

We conducted semi-structured interviews with 19 peer specialists. Each interview followed the four-stage procedure shown in Figure 1. We initially planned to conduct the interviews in pairs to support discussion around participants' experiences and perspectives on AI safety. However, differences in participants' availability, shift-work schedules, and last-minute scheduling changes meant that some participants could not be paired as planned. As a result, 10 participants took part in five paired interviews, while 9 were interviewed individually. The two formats provided somewhat different opportunities for eliciting participants' perspectives. Paired interviews allowed participants to hear and respond to one another's experiences, while individual interviews provided more space for participants to elaborate on their own experiences and practical judgments. Interviews were conducted by three facilitators from the research team, lasted approximately 90 minutes, and were held over Zoom. The study protocol was approved by our institution's Institutional Review Board (IRB), and all participants provided informed consent before participating.

Stage 1: Introduction and Background (5 minutes). Participants described their peer-support backgrounds, roles at their organizations, and prior experiences with AI tools, including their use in peer-support work.

Stage 2: PeerAlly Familiarization and Grounding (10–15 minutes). Building on their pre-interview familiarization with PeerAlly, participants explored the system hands-on to establish a shared reference point for discussing AI safety.

Stage 3: AI Risk Scenario Card Elicitation (40–45 minutes). Participants engaged with the five AI risk scenario cards described above. For each scenario, we elicited participants' concerns and responses, the contextual and competing considerations shaping their judgments, how these judgments might vary across service users and situations, and what support they would need to navigate these risks in practice.

Stage 4: Follow-up Discussion (20–25 minutes). We asked participants what safe AI use means in peer support, what peer specialists should consider when using AI, and what support would help them navigate AI-related risks in practice. We then discussed how such support could be maintained and evolved organizationally over time, including participants' perspectives on existing organizational practices and processes for sustaining support.

3.5 Data Analysis

We analyzed interview transcripts using reflexive thematic analysis [17, 18]. During initial coding, three researchers repeatedly read the transcripts, took analytic notes, and coded participants' accounts of AI-related concerns, practical judgments, practices, and envisioned forms of support for navigating AI safety. The researchers met regularly throughout the analysis to compare interpretations, discuss emerging patterns, revisit relevant excerpts, and iteratively refine and reorganize the codes. Through successive rounds of coding and discussion, we grouped related codes into candidate themes, examined relationships among them, and iteratively refined their boundaries and organization. We then synthesized the themes into higher-level thematic categories used to structure the findings. We did not calculate inter-rater reliability, as collaborative interpretation and repeated engagement with the data were central to developing and refining our themes [18, 57]. Through this iterative process, we developed a set of first-level themes, which were clustered into 17 second-level themes and ultimately synthesized into three overarching themes that structure our findings: (1) identifying what support for AI safety should enable in relational practice (Section 4.1); (2) supporting AI safety as relational practice unfolds (Section 4.2); and (3) building organizational capacity to sustain and evolve AI safety (Section 4.3).

3.5.1 Positionality Statement. Our disciplinary backgrounds and prior research experience informed both the design of this study and our interpretation of the data. The research team brought expertise in human-computer interaction, computer science and AI, responsible AI, and mental and behavioral health research, as well as prior research experience with peer specialists and community behavioral health settings. These experiences provided familiarity with the values and practices of peer support while also shaping the perspectives through which we approached questions of AI safety. Building on prior HCI research with frontline workers in sensitive and high-stakes community and behavioral health contexts [48, 74], we were attentive to how our research design and our own perspectives might shape participants' engagement with and interpretation of AI safety.

Members of the research team developed PeerAlly and designed the scenario cards, which could shape which risks became salient

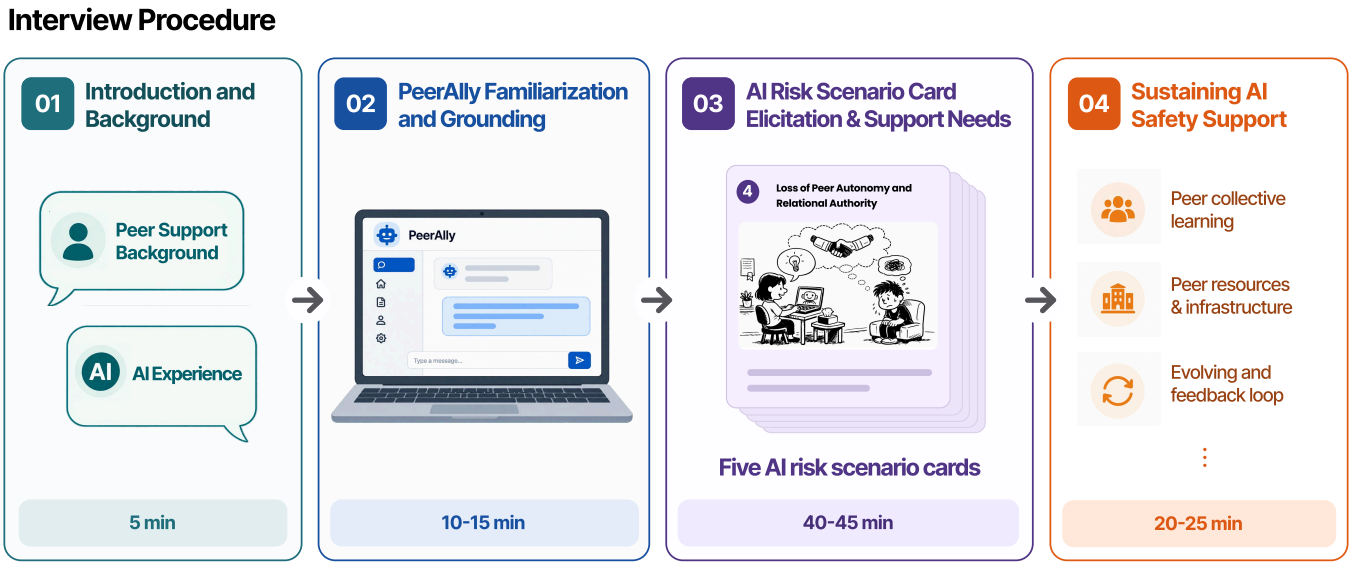


Figure 1: Overview of the four-stage interview procedure. Participants (1) discussed their peer-support backgrounds and prior AI experiences, (2) familiarized themselves with PeerAlly through hands-on exploration (see Appendix A), (3) engaged with five AI risk scenario cards to reason about potential safety concerns and support needs, and (4) reflected on how such support could be sustained through collective learning, organizational resources and infrastructure, and ongoing evolution and feedback. The full set of scenario cards is provided in Appendix B.

and participants' willingness to critique the system. To encourage open discussion, we explicitly invited participants to speak freely about potential problems with using PeerAlly. We also treated the scenario cards as starting points rather than boundaries for discussion, encouraging participants to draw on their own experiences and reflect more broadly on AI safety judgments and support needs beyond the depicted situations.

4 Findings

Participants brought their existing peer-support experience, along with varying levels of prior experience with LLMs, to their reasoning about AI safety. Several described using AI to draft or refine work-related communication and documentation, organize community resources, plan group activities, and search for resources relevant to service users' needs. Some participants had already considered safety in their use of LLMs, for example, by providing only non-identifying information, such as a service user's county or presenting issue, when using AI to search for resources. These prior experiences provided a practice-based starting point from which participants reasoned about the scenarios and the support they would need to navigate AI safety in peer-support practice.

We organize our findings around three themes: (1) what support for AI safety should enable in relational practice (Section 4.1); (2) supporting AI safety as relational practice unfolds (Section 4.2); and (3) building organizational capacity to sustain and evolve AI safety (Section 4.3).

4.1 Identifying What Support for AI Safety Should Enable in Relational Practice

We identified three interconnected qualities that participants sought in support for AI safety: (i) grounding general AI risks in the values and realities of peer-support practice, (ii) providing actionable guidance that could be interpreted and adapted through peer specialists' existing practical knowledge, and (iii) remaining relevant as safety considerations arose at different points in their relationships with service users.

Participants emphasized the need to ground general AI risks in peer-support practice. P19 highlighted the importance of making risks relevant to peer specialists' work and the people they support: *"The risks should be relevant to the practice [...] why it matters to the center member, like the person that I'm trying to help."* Similarly, P3 called for specific, actionable warnings rather than generic cautions: *"[Not] just like a little warning, like, 'Be careful' [...] 'There may be a risk.' Because when you say a risk, I sit here and I'm like, 'Okay, well, what kind of risk?'"* Guidance therefore needed to go beyond identifying risks to help peer specialists understand how those risks might manifest in peer-support practice and what they could mean for particular service users.

Participants also emphasized that AI safety guidance needed to remain adaptable to particular service users and situations. P17 connected this flexibility to how peer specialists already put practical guidance into action, describing *"learning how to navigate specific situations and taking what you learned and putting it into practice for the array of situations that you may encounter."* P13 similarly emphasized the expertise involved in making judgments: *"That's what we do in this field anyway. If you're a peer, you're kind of trained to think*

on your feet. You're relying on experience and lived experiences, so it's our job to like kind of freestyle it anyway." P15 valued actionable guidance that still left room for practical judgment: *"[Tell us] what we should do, or what you recommend we do. But there's always going to be a gray area."* Participants thus sought direction that left room to respond to service users' perspectives and preferences and to adapt support to the particular relationship and situation, rather than prescribing universal responses.

Participants emphasized that support needed to remain relevant across different moments of AI's involvement in relationships with service users—from obtaining initial consent, to generating and sharing support, to checking in on service users' experiences afterward. P18 explained, *"I like a guidance based on a step-by-step peer support workflow. We can really relate it to our own situations and work, so it feels much more practical. [...] [I like] to go one step at a time, where one step naturally leads to the next."* Accounts such as P18's illustrate how participants envisioned guidance accompanying their work as distinct safety considerations arose throughout their relationships with service users. Finding 4.2 examines how these considerations, judgments, and support needs varied across key moments of relational practice.

4.2 Supporting AI Safety as Relational Practice Unfolds

Across participants' accounts, we identified five recurring moments at which different AI-related safety considerations and practical judgments became salient as relationships with service users unfolded: (i) preparing for relationally grounded AI use, (ii) negotiating AI's entry into the relationship, (iii) generating support with AI while staying attuned to service users, (iv) sharing AI outputs with the person and relationship in mind, and (v) reflecting on AI's impact on service users and peer-support relationships. These moments reflect distinct points in relational practice at which different judgments about AI's involvement became salient, rather than simply stages before, during, and after AI use. Importantly, participants anticipated that what they learned about service users' experiences of AI involvement could inform subsequent safety judgments, prompting them to revisit earlier judgments about whether AI should remain involved and how it should be used. Figure 2 provides an overview of these five moments and how learning from later moments can reopen earlier safety judgments.

Across these moments, participants articulated different safety goals and practices, along with the values, tensions, and practical judgments involved in pursuing them. Their accounts also surfaced ways that support could scaffold their judgments at different points in relational practice. The following sections examine *when* these judgments arose and *how* support could scaffold them.

4.2.1 Moment 1. Preparing for relationally grounded AI use.

Participants emphasized that preparing to bring AI safely into their work with service users involved more than understanding AI's capabilities, limitations, and risks. It also required grounding their approach in peer-support values and clarifying AI's role and boundaries within peer-support practice.

Ground AI use through a peer-support lens. Participants emphasized that peer specialists should center their peer support values before incorporating AI into their work. P19 articulated this directly: *"I think it is important for them to be grounded in peer support values before the use of AI. You want to make sure probably that person is in the right headspace, in the right mindset before they're helping other people. Let alone use AI."* P17 similarly emphasized that peer-support values should come first: *"I would highly emphasize learning our organization's core peer-support values. I would say that would come first. [...] just making sure the relationship that we have with the members is the biggest priority."* Rather than relying on AI knowledge as the starting point, participants thus situated readiness for AI use within a commitment to peer-support values and relationships with service users.

Scaffold reflection on AI's role and boundaries in peer-support practice. Participants also emphasized the importance of guidance that encouraged peer specialists to reflect on and clarify AI's role in relation to their own expertise and responsibilities before incorporating it into their work with service users. P5 described guidance that promoted viewing AI as a supplementary resource rather than a substitute for peer practice: *"This is supposed to be a tool, not your script necessarily. Like you're not supposed to be copying it word for word, bar for bar. You're using this to help you better serve the person who's coming to you for help."*

This guidance could help peer specialists consider how AI might support their work without replacing their own judgment or role. It also preserved their responsibility for interpreting and acting on AI-generated information. As P16 explained, *"Use bits and pieces, but come from using what you've learned, using what you know, incorporated with the AI information, not just solely based on the AI information [...] using AI to kind of guide the conversation, but it will be solely on you."* Collectively, these accounts point to the need for guidance that grounds AI's role in peer-support values while maintaining peer specialists' judgment, role, and responsibility for its use.

4.2.2 Moment 2. Negotiating AI's entry into the relationship.

Once peer specialists were prepared to engage with AI, participants emphasized that bringing AI into interactions with a service user should not be done in a standardized fashion or treated as a decision for peer specialists to make unilaterally. They instead described the need to consider *whether*, *when*, and *how* to raise the possibility of AI use with a particular service user, while attending to the person's concerns, preferences, and right to decline its involvement.

Support shared decisions about whether to involve AI. Participants emphasized that decisions about whether to involve AI should not rest solely with peer specialists, but should be discussed with service users in ways that preserve their awareness and ability to decline. P7 suggested *"let[ting] them know before [...] that we'd be using an AI tool"* and proceeding only *"if they're okay with that."* P6 similarly emphasized that knowing ahead of time allowed service users to *"choose a yes or no"* rather than being *"blindsided."*

At the same time, participants described multiple considerations that could inform how peer specialists approached this conversation. P6 envisioned using AI to help members who were hesitant to leave the hospital explore community resources and become more comfortable with technology. Yet they also anticipated difficulties introducing AI to members who experienced paranoia, particularly when it involved *"AI sending the messages and stuff like*

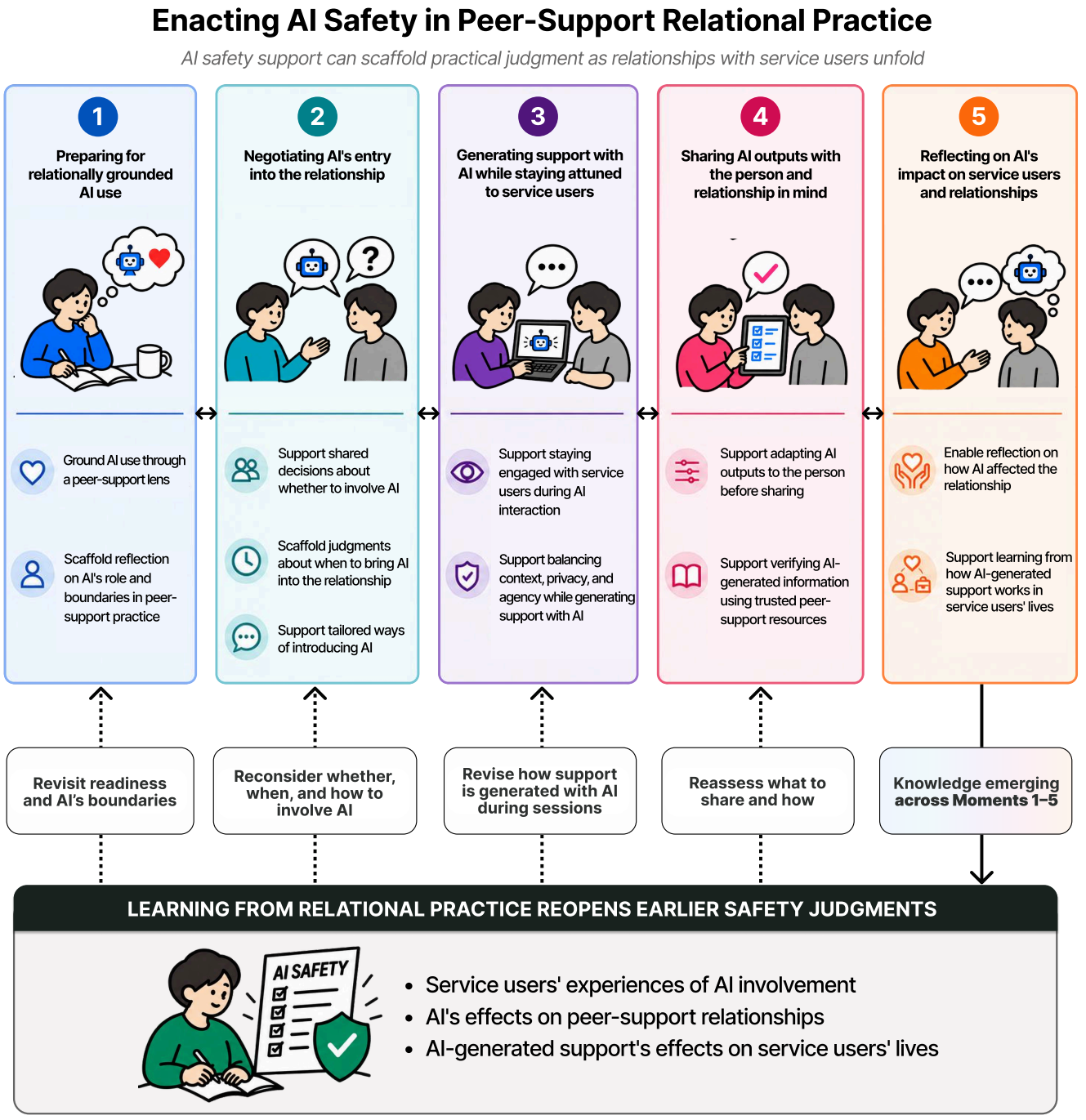


Figure 2: Overview of five moments of AI-supported peer-support practice, showing how support for AI safety can scaffold different practical judgments and actions as relationships with service users unfold.

that." Other participants similarly described technology and privacy concerns, explaining that some service users feared that sensitive information, such as their criminal or substance use histories, might be shared with city authorities, immigration authorities, or other service providers. Participants also worried that introducing AI could lead some service users to rely on it independently and without sufficient scrutiny, particularly given its potential to produce misleading or inappropriate information. P1, for example, raised concerns that service users might *"use AI to diagnose themselves"* or *"use AI as a way to kind of live life."* Introducing AI could therefore

reshape the peer specialist's role, requiring them to help service users understand AI's limitations and make an informed choice about its involvement. Together, these accounts show that deciding whether to involve AI was not simply a matter of obtaining consent, but of negotiating its potential benefits, risks, and implications with a particular service user while preserving their ability to decline its involvement.

Scaffold judgments about when to bring AI into the relationship. Participants emphasized that deciding when to introduce AI should not be treated as a routine or automatic step. Instead, it requires careful consideration of the service user's needs, the stage of the relationship, and the kind of support required at that moment. For some, establishing rapport with a service user needed to come first. P17 strongly emphasized *"building that rapport with individuals first. Because at first you don't need to have all the answers. You just need to connect with them as a person."* Yet the same participant qualified this preference not to introduce AI *"in the first session"* when a service user was *"very, very adamant that they needed a resource."*

In contrast, P14 described involving AI immediately when a service user needed practical assistance: *"We have to find a solution, [...] And I think AI can help us with resources a lot quicker."* These differing accounts show that the appropriate timing of AI's entry was itself a practical judgment, shaped by what the service user needed from the encounter and the state of the relationship.

Support tailored ways of introducing AI. Participants emphasized that how AI was introduced would matter for the relationship with the service user. P3, for example, described service users who were *"nervous about AI because they don't trust it"* and explicitly wanted guidance on what to say in response: *"What can we say to them to reassure them that this is safe? [...] What words we can use to say to reassure them?"* P12 similarly described the need to explain how personal information would be handled and protected when speaking with service users who distrusted AI, while introducing the technology more openly to those already comfortable with it. P4 emphasized the importance of accessible communication, *"making sure that what I was trying to explain was at a level that the majority of individuals would be able to understand."* Collectively, these experiences suggest the need for guidance that could help peer specialists consider how to introduce AI to a particular service user, as what to explain and how to explain it could vary depending on the person's concerns, familiarity with AI, and level of understanding.

4.2.3 Moment 3. Generating support with AI while staying attuned to service users.

Once peer specialists and service users decided to involve AI in their relationship, participants emphasized the need for support in judging how to use it while remaining attentive to the service user and how to represent the person's needs to AI without compromising their privacy, context, or agency.

Support staying engaged with service users during AI interaction. Participants emphasized the need for guidance that could help them remain present while interacting with AI. P5 imagined *"that screen between us"* while a service user discussed sensitive experiences, warning that sustained attention to the screen could create *"that risk of losing the trust."* Participants wanted support that could help them manage this risk by maintaining and redirecting their attention toward the service user while using AI. P16 suggested that guidance might prompt peer specialists, *"If you do need to use the AI [...] continue to make eye contact. And once you put it in there, I would engage with the person. And not solely based on AI, but using your own personal reflective empathy [...] leading that conversation."* P19 similarly suggested that guidance could intervene when AI interaction began drawing attention away from the person: *"You've been having repetitive dialog with chat for X amount of time or multiple back and forth responses, maybe engage with the client or person served."* These accounts highlight attention itself as a safety-relevant concern: even when AI was used to support the service user, interacting with it could compete with the relational presence on which peer support depends.

Support balancing context, privacy, and agency while generating support with AI. Participants described a tension between providing enough context for AI to generate relevant support and protecting service users' private information. P12 explained, *"It depends on the call. You can't just go and just put the basic information [...] you're gonna fail every time,"* while also cautioning, *"You don't want to use information [...] you got no business [using]. Focus on what you're there for rather than putting their identity and confidential information in jeopardy."* Participants wanted guidance that could help them judge what contextual information was necessary for a particular support need, rather than prescribing a fixed set of information to provide to AI. They also considered how service users could participate in shaping how their needs were represented. P4 suggested *"having reminders about being able to use it in collaboration with the person served [...] giving them that power to be able to contribute... as well."* P8 similarly envisioned inviting a service user to provide details and consider together *"what would work for you, what would help you,"* describing AI use as *"more of a group effort"* and proposing, *"let's do it together."* For P8, this meant remembering that peer specialists were *"here to support and not direct"* and *"let[ting] them take the lead"* when using AI together. Together, these accounts show that representing a service user to AI involved balancing competing demands: providing enough context for relevant support, withholding information that could compromise privacy, and preserving the service user's agency in how their needs were represented.

4.2.4 Moment 4. Sharing AI outputs with the person and relationship in mind.

Participants emphasized that generating a seemingly useful AI response did not mean it could be automatically shared with a service user. Before bringing AI-generated material into the relationship, participants emphasized the need to judge whether and how it should be adapted for the particular person and whether the information was trustworthy enough to share or act on.

Support adapting AI outputs to the person before sharing. Participants emphasized that sharing AI-generated material required considering both the boundaries of their role and the particular service user's circumstances and sensitivities. P18 described the need to consider the service user's particular *"case"* and *"triggers that can come up,"* suggesting that peer specialists provide a *"warning"* before sharing potentially triggering material. Other participants similarly described reviewing AI outputs for content

that might reproduce bias, stigmatize the service user, or respond inappropriately to emotions inferred from the prompt. Participants wanted guidance that could help them determine what AI-generated material was appropriate to share and how it should be filtered, reframed, or contextualized for a particular person and relationship.

Support verifying AI-generated information using trusted peer-support resources. Participants described the importance of verifying AI-generated information using knowledge and support resources already embedded in peer-support practice. P2 emphasized verifying information before sharing it with a service user: *"Peer specialists have the right to verify that information. Before we give someone a resource, we make sure that the resource is actually available right now."* Participants also described consulting organizational and local resource databases to determine, *"Is this a legitimate resource in the county? [...] Has it closed down, or is it even real?"* P7 similarly suggested drawing on existing resources, explaining, *"In our department, we have a binder with different resources for the different counties that we serve. So we can kind of cross-check with that."* P9, P10, P11, and P18 also referred to organizational resource libraries and local resource databases that peer specialists already used. Verification could further draw on other peers' knowledge. P10 suggested, *"Just get another wellness mentor and say, 'Hey, does this make sense?'"* Verification was therefore not imagined as a generic fact-checking task, but as one grounded in the local resources and collective knowledge through which peer specialists already established what information was trustworthy and actionable.

4.2.5 *Moment 5. Reflecting on AI's impact on service users and peer-support relationships.*

Participants emphasized that judgments about safe AI use should not end with a session. Reflection could reveal how service users experienced AI's involvement, how it affected the peer-support relationship, and how the support generated with AI subsequently worked in their lives. Participants described how learning from these experiences could inform revisions to earlier judgments and adaptations to future AI use.

Enable reflection on how AI affected the peer-support relationship. Participants emphasized a need to reflect with service users on how AI's involvement had affected the interaction and whether anything about its use should change. P18 described asking after a session, *"How do you feel about this?"* or *"How do you think PeerAlly helped you?"* and using the service user's response to consider whether and how to continue using AI with that person. P17 similarly emphasized keeping that decision open to revision: *"If you no longer want me to use this while I'm working with you, you just let me know. There's absolutely nothing wrong with that. It's pretty much what you feel comfortable with."* These accounts point to support for prompting reflection on the service user's experience of AI during the interaction and carrying that feedback into subsequent decisions about AI involvement. Feedback on how service users experienced AI could reopen earlier judgments about whether AI should remain involved in the relationship and, if so, how it should be introduced and used in subsequent encounters. In this way, participants anticipated that how AI affected the service user and relationship could itself become safety-relevant knowledge for subsequent judgments about AI involvement.

Support learning from how AI-generated support works in service users' lives. Participants emphasized learning what happened after service users acted on information or resources generated with AI. Information that initially appeared appropriate could prove unsuitable because of circumstances unknown when it was generated or shared. P12 described recommending a seemingly relevant resource, only to hear from a service user, *"Oh, I can't go there. They have denied me. They treated me really poorly in the past. [...] Find me another resource."* As P12 explained, *"you kind of have to go resource by resource and see what works for the person we are serving."* Participants therefore valued support for following up on these downstream experiences. P8 described asking service users, *"So how did this go?"* to learn how previously provided information or resources had worked in their lives. P19 characterized this process as *"almost like a trial and error,"* explaining, *"I also learn from it too."* P19 further reflected that they would use AI *"more or less"* depending on whether it benefited the people they supported. These accounts suggest how learning from service users' experiences with recommended resources could inform subsequent AI-supported practice. Learning how that guidance worked in service users' lives could reopen earlier judgments about which resources to generate, what information to provide to AI, and what outputs were appropriate to share.

Together, these accounts suggest that reflection could generate knowledge that feeds back into judgments across earlier moments, including whether and how to involve AI, what context to provide when generating support, and which outputs to adapt or share. Supporting AI safety therefore involves helping peer specialists revisit these judgments as they learn more about a particular service user, relationship, and situation. This need to revisit judgments raised a broader question beyond individual interactions: how organizations could sustain such support over time and enable what peer specialists learned through practice to inform future guidance.

4.3 Building Organizational Capacity to Sustain and Evolve AI Safety

Participants emphasized that learning from AI-supported practice should extend beyond individual peer-support relationships. They drew on existing structures through which peer-support knowledge is learned, shared, and reinforced to envision how AI safety could be sustained and evolve over time. Specifically, they described how individual experiences with AI could become collective peer knowledge, how AI safety could be integrated into existing structures for learning and reinforcing peer-support practice, and how emerging practice knowledge could inform the ongoing evolution of safety guidance.

4.3.1 *Turning individual experiences into collective peer learning.*

Participants wanted ways to turn peer specialists' individual experiences with AI into shared organizational knowledge. P3 described current AI learning as largely self-directed: *"I haven't been taught [by] anyone in my center personally. So I'm just learning it myself, but I've learned a lot in a short amount of time by using it so often."* Participants wanted opportunities to bring this experiential learning back to peers, forming collective rather than siloed knowledge. P10 proposed *"one introductory group"* where peer specialists could first try AI with a hypothetical service user, followed by another group

"a month later" where peers could share, *"This is my experience. I had this problem,"* and *"This is how I solved that problem."* Participants described how this practice could expand beyond individual organizations. P13 described the Peer Summit, where peers exchange *"information, situations, scenarios"* and *"we all relay information to each other and adapt and learn."* They described bringing what was learned in these broader communities back into local organizations, allowing experiences and emerging strategies around AI safety to circulate across the peer community.

Participants also described how knowledge emerging through AI use could be incorporated into shared organizational resources. P11 also saw value in enabling peer specialists to contribute what they learned about service users' needs and responses, helping colleagues tailor subsequent support and consider relevant AI safety concerns. P17 described taking resources identified through PeerAlly and *"implement[ing] them into the resource library [...] or we can bring it into conversations that other staff can kind of learn about, to benefit members."* Collective learning could therefore involve not only exchanging experiences among peers, but also accumulating what peer specialists learned through AI use in shared resources that others could draw on.

4.3.2 Integrating AI safety into how peer-support practice is already learned and reinforced. Participants did not describe AI safety as a separate technical competency to be learned alongside peer-support practice. Instead, they envisioned it as part of the existing structures through which peer-support values, norms, and expertise were already learned and reinforced. P12 described existing *"quarterly trainings"* on *"ethics, psych rehab, mental health, substance use, person served language, [and] person centered approach,"* suggesting that organizations could *"add this training into that"* and *"train annually your staff to refresh every year."* P4 further cautioned that introducing AI while newcomers were still learning what it meant to practice as peers could *"feel contradictory,"* explaining, *"We're really owning it on the values of peer support, and then telling them, 'Oh, but you can also use this online tool' and have the risk of disconnecting peer support. It doesn't really align."*

Participants extended this integration beyond formal training to organizational resources and channels that peer specialists already used, including resource libraries, internal platforms, supervision and communication channels, and policies. P17, for example, wanted a section on AI within their center's *"policy and procedure manual"* that provided support *"related specifically to our values."* Together, these accounts position AI safety learning within the existing structures through which peer-support values and expectations are learned, reinforced, and carried into everyday practice.

4.3.3 Evolving support for AI safety through emerging practice knowledge. Participants emphasized that initial guidance could not anticipate all of the situations that would emerge as peer specialists gained experience with AI. They suggested that experiences from practice could serve as a continuing source for revising guidance over time. P5 proposed collecting feedback at multiple points after guidance was introduced: *"Maybe a month feedback first of How are you liking this?' or Have you been using this?' and then like six months, a year, things like that, just in case if the peers are realizing like maybe there's more information that should be on certain questions or certain information that should be on there."*

Participants also emphasized incorporating perspectives from those differently involved in and affected by AI-supported practice. P16 explained, *"I think it should be collectively all the peers, the members, the staff, the everybody, the peer specialists. Everyone should be responsible and provide feedback. We want feedback from the members. We want feedback from staff. We want feedback from our peers, our coworkers."* Participants further recognized that this learning process would need to continue as changes in AI introduced new or previously unanticipated risks into practice. P4 emphasized keeping guidance *"up to date with current AI and technology advancements,"* because such changes could *"increase or reveal new risks that peer specialists using AI could run into."* Rather than treating guidance as fixed, participants thus envisioned an ongoing process in which emerging experiences with AI informed how new and evolving risks could be understood and addressed in peer-support practice.

5 Discussion

5.1 Conceptualizing Support for AI Safety in Relational Practice

Prior HCI research has emphasized that AI safety is shaped by the social and organizational contexts in which AI is embedded and by whose knowledge, values, and experiences are centered [12, 29, 46]. Our findings show that determining what AI safety requires in relational practice involves two intertwined challenges. First, what constitutes safe AI use can vary across people, situations, and relationships, requiring peer specialists to exercise practical judgment rather than apply general guidance uniformly. Second, these judgments cannot be treated as settled. As AI becomes involved in interactions with service users, its effects on people and relationships can themselves become safety-relevant knowledge, requiring earlier judgments to be revisited. However, existing support for AI safety among those working with AI has largely taken the form of guidance for safer AI use, providing information about AI capabilities and risks, recommended practices, and harm-mitigation strategies [3, 7, 9, 26, 52]. Such guidance can establish important safety objectives and practices, but offers less scaffolding for making practical judgments for particular people and relationships and revisiting those judgments as workers learn how AI involvement affects people and relationships over time.

This motivates an alternative conceptualization of support for AI safety in relational practice. Drawing on prior accounts of enactment as the interpretation and translation of broader principles into situated practice [10, 16, 66], we propose conceptualizing such support as scaffolding the *enactment of AI safety*. We conceptualize AI safety enactment as a recursive process involving three interconnected components: (i) interpreting general safety objectives in relation to a particular person and relationship, (ii) translating that interpretation into action, and (iii) revisiting earlier safety judgments as workers learn how AI's involvement affects people and relationships over time. Conceptualizing support around this process, in turn, shifts how such support is understood in two ways: from providing predefined guidance to scaffolding practical judgment, and from treating context as already given to recognizing relational practice as a source of evolving safety-relevant knowledge.

From providing predefined guidance to scaffolding practical judgment for AI safety. Existing AI safety guidance can establish important objectives and boundaries, but, as prior work recognizes, general guidance may not fully account for what safety requires in particular domains [26]. Our findings show how this limitation manifests in relational practice. Safety concerns not only the properties of AI outputs, but also how AI involvement affects a particular person and relationship. Evaluating an AI response, for example, would involve more than assessing its accuracy. Peer specialists described how they would assess whether it fit a service user's circumstances, what should be filtered or reframed given the person's sensitivities, and whether sharing it would remain consistent with peer-role boundaries. Even whether and when to involve AI depended on the person's preferences, the state of the relationship, and the immediate support need. AI safety in relational practice is therefore not simply achieved by applying guidance; it is enacted through workers' interpretation of what broader principles require for a particular person, relationship, and moment. This situated interpretation can be understood through Schön's notion of *reflection-in-action*, in which practitioners reflect within ongoing action as they respond to the contingencies of practice [86, 87]. In AI-supported relational practice, this lens helps explain why support for AI safety should scaffold workers' practical judgment as they interpret and adjust what safety requires while interactions unfold. This reframes support for AI safety from providing guidance for workers to apply toward scaffolding the practical judgment needed to determine what safe action requires in a particular relational encounter.

From treating context as given to treating relational practice as a source of evolving safety-relevant knowledge. Prior HCI research has emphasized learning from workers' experiences as technologies and risks evolve [92, 116]. Our findings highlight relational practice as a source of this learning. Participants' accounts suggest that some safety-relevant knowledge may emerge only as peer specialists and service users interact and learn how AI's involvement affects the person and relationship. Participants described the need to make different judgments as they prepare to use AI, negotiate whether and when it should enter a relationship, use it while supporting a service user, adapt and share its outputs, and reflect on its impact afterward. Across these moments, peer specialists can learn more about service users, observe their responses to AI's involvement, and see how AI-generated support operates in their lives.

Importantly, how AI is involved in these interactions can itself shape the relationship in which subsequent safety judgments are made. For example, how AI is introduced or used may affect a service user's trust, comfort, or willingness to engage, changing the relational conditions that inform later judgments about AI involvement. These moments are interconnected rather than fixed steps. What peer specialists and service users learn through later interactions can reopen earlier judgments about whether AI should remain involved, what role it should play, or what forms of support are appropriate. Revisiting these earlier judgments can be understood through Schön's notion of *reflection-on-action*, in which reflection on prior experience informs future practice [86, 87]. In AI-supported relational practice, however, what workers reflect on includes how AI's involvement may itself have shaped the relationship, generating safety-relevant knowledge that can change what safe involvement requires in subsequent interactions. The recursive character of AI safety enactment therefore arises not only from reflection on experience, but from the possibility that AI involvement itself changes or reveals the relational conditions against which subsequent safety judgments must be made. Our findings thus suggest that relational practice could serve as a site where safety-relevant knowledge not available at the outset emerges and accumulates.

Drawing on these two shifts, we contribute a conceptualization of support for AI safety in relational practice as scaffolding an ongoing process of enactment in which workers exercise practical judgment and revisit earlier judgments as new safety-relevant knowledge emerges through interaction. We encourage HCI researchers to investigate how this conceptual lens may transfer to other domains where relational practice is central, including mental health care and social services, while attending to how different relational norms and forms of work may shape what safety requires.

Our findings also caution against treating AI integration as the default goal of support for AI safety. Whether AI should be involved, in what form, and at what moments is itself a safety judgment that should remain open. The support mechanisms we propose are therefore intended not only to enable safer AI use, but also to support decisions to limit, defer, or reject AI involvement when it conflicts with relational values or safety needs.

5.2 Design Implications for Enacting and Sustaining AI Safety in Relational Practice

Building on the two shifts above, we derive two implications for designing support for AI safety in relational practice.

Organize and deliver support for AI safety around relational moments. Because AI safety is enacted through practical judgment, support should help workers reason at the relational moments when such judgment is required. Existing guidance and resources for AI safety are often organized around stages of AI use, phases of development and deployment, or predefined categories of risk such as privacy, accuracy, and oversight [3, 7, 9, 26, 52]. These structures identify relevant safety concerns but provide less guidance about when and how workers engaged in relational practice should interpret and act on them within a particular relationship. We therefore propose organizing support for AI safety around *relational moments*: points at which AI's involvement requires workers to determine what safe action requires for a particular person and relationship. Relational moments are not simply contextual factors that influence whether an AI use is safe. Rather, they mark points in the unfolding relationship at which workers must make or revisit a safety-relevant judgment about how AI should participate in that relationship. Contextual factors—such as a person's preferences, circumstances, or prior experiences—inform these judgments, while relational moments identify when and around what relational action those judgments become necessary.

Our findings in peer support illustrate how this design principle might take shape. As discussed in Section 4.2, participants reasoned about different practical judgments across relational moments—from preparing for AI use and negotiating AI's entry into

the relationship, to generating support while staying attuned to service users, sharing AI outputs with the person and relationship in mind, and reflecting on AI's impact afterward. Other relational settings may surface different relational moments at which safety-relevant judgments about AI become salient, depending on their relational norms and forms of work. Identifying these moments could similarly inform when and how support for AI safety is organized and delivered in those settings.

Systems could operationalize this approach by allowing workers to indicate when support is needed, or by using contextual cues to offer timely support, as explored in prior work on just-in-time objectives and proactive AI assistance [24, 50]. As a scheduled session approaches, for example, the system could offer preparatory information to a worker planning to introduce AI, helping them consider how to respond if the service user expresses interest, hesitation, or refusal, and how each response might shape AI's role in the relationship. During a session, actions such as entering a service user's circumstances or searching for support resources could cue subtle or on-demand prompts that surface relational considerations for workers to attend to, such as maintaining attention to the service user, checking in about their comfort with AI's involvement, and preserving their agency. Rather than assessing the state of the relationship or determining an appropriate response, these prompts would scaffold workers' attention to considerations that may be relevant to their own situated judgment. The role of AI would thus be to serve as a reflective aid that surfaces questions and possible contingencies, while leaving workers and service users to determine what the relationship requires as their interactions unfold.

Provide mechanisms for reflection and collective learning to evolve and sustain support for AI safety. Because support for AI safety should treat relational practice as a source of evolving safety-relevant knowledge, support should enable workers to revisit earlier judgments and carry that learning forward. In peer support, our findings illustrate how reflection on experiences across relational moments can feed emerging knowledge back into judgments made at earlier moments. Prior HCI research has conceptualized reflection support as helping people recognize breakdowns in experience, inquire into what occurred, and translate new understandings into changed perspectives or actions [11]. Other work has shown how reflection-support technologies can help people revisit experiences over time and reinterpret them through conversation, comparison, and discovery [14]. Building on these approaches, HCI designers could create mechanisms for workers and service users to reflect on how AI's involvement affected the person and relationship and carry that learning into subsequent encounters. For example, post-session reflection tools could help workers record changes in a service user's preferences, significant responses to AI-generated support, and relational events that might require a different approach in the future. With the service user's knowledge and consent, the system could bring these reflections forward when preparing for a later encounter—for example, reminding the worker that the person previously preferred AI-generated information to be shortened, reframed, or avoided in particular situations. These records should not function as fixed profiles, but as starting points for reconsideration as the person's preferences, needs, and circumstances change.

To sustain and adapt such support over time, opportunities for collective learning could be embedded within existing practices and structures within and across organizations. Building on mechanisms for articulating workplace experiences and supporting collaborative reflection [79, 80], relational experiences encountered by individual workers could be shared through colleague networks, supervision, case discussions, or organizational learning systems, allowing recurring concerns and responses to become collective safety knowledge. For example, accounts of service users feeling excluded during AI use could inform new training scenarios and guidance on maintaining their participation. This knowledge could inform revisions not only to AI systems, but also to organizational safety objectives, policy guidance, supervision practices, and resource libraries. It could also potentially circulate across communities of practice. Connecting these feedback mechanisms to existing structures could allow workers to contribute through familiar channels, learn from others' experiences, and carry emerging collective knowledge back into their own practice. However, these mechanisms should remain under the control of workers and service users, minimizing documentation burdens and identifiable information while preserving the relational context of safety judgments.

6 Limitations and Future Work

First, given our focus on supporting workers in navigating AI safety, this study centered on peer specialists' perspectives. Complementary work with service users could further examine how they would want to participate in AI-supported peer support. For example, future work could explore their preferences for how peer specialists communicate about AI, involve them in generating and reviewing AI-supported content, and reflect with them on AI's involvement in their relationships. These perspectives could further inform the design and refinement of support for workers navigating AI safety.

Second, as researcher-designed elicitation materials, the scenarios and their visual representations foregrounded particular AI risks and understandings of peer-support interactions, which may have shaped participants' responses. Although the cards served as starting points rather than boundaries for discussion, enabling participants to draw on their experiences and discuss broader concerns, our findings do not exhaust the AI safety concerns that may emerge in everyday peer-support practice. Future work could co-design such materials with peer specialists to surface risks beyond those anticipated by the research team.

Third, using PeerAlly as an elicitation material, our study explored how participants envisioned navigating AI safety and what support they would need. We did not examine its deployment or longitudinal use with service users. Future longitudinal studies could investigate how these anticipated judgments and support needs unfold in everyday interactions and evaluate how the proposed support helps workers enact and sustain AI safety.

Finally, our study involved peer specialists in the United States and may not capture variation across organizational, cultural, and geographic contexts. Future work should examine a broader range of peer-support organizations and settings, including those outside the United States.

7 Conclusion

This study introduces a conceptualization of support for AI safety in relational practice as scaffolding the *enactment of AI safety*. Through semi-structured interviews with 19 peer specialists, we characterize what support for AI safety should enable, when and how it should scaffold practical judgment, and how such support can be sustained and evolved over time. Our findings suggest that support for AI safety should move beyond predefined guidance to scaffold practical judgment for particular people and relationships while enabling earlier judgments to be revisited as relationships unfold. We conceptualize this enactment as a recursive process and discuss design opportunities for organizing support around relational moments and enabling reflection and collective learning to sustain and evolve such support over time.

References

[1] American Psychological Association. 2024. APA's AI Tool Guide for Practitioners. American Psychological Association. https://www.apaservices.org/practice/business/technology/tech-101/evaluating-artificial-intelligence-tool

[2] American Psychological Association. 2025. Ethical Guidance for AI in the Professional Practice of Health Service Psychology. American Psychological Association. Updated July 2025. https://www.apa.org/topics/artificial-intelligence-machine-learning/ethical-guidance-professional-practice.pdf

[3] American Psychological Association. 2025. Health Advisory: Use of Generative AI Chatbots and Wellness Applications for Mental Health. American Psychological Association. Created November 2025. https://www.apa.org/topics/artificial-intelligence-machine-learning/health-advisory-chatbots-wellness-apps

[4] American University. [n. d.]. University Guidance on the Responsible Use of AI. Office of Information Technology. https://www.american.edu/oit/security/responsible-use-of-ai.cfm

[5] Dario Amodei, Chris Olah, Jacob Steinhardt, Paul Christiano, John Schulman, and Dan Mané. 2016. Concrete problems in AI safety. *arXiv preprint arXiv:1606.06565* (2016).

[6] Oren Asman, John Torous, and Amir Tal. 2025. Responsible design, integration, and use of generative AI in mental health. *JMIR mental health* 12, 1 (2025), e70439.

[7] Australian Government, Digital Transformation Agency. 2026. Guidance for the Artificial Intelligence Impact Assessment Tool: Reliability and Safety. Australian Government. https://www.digital.gov.au/ai/impact-assessment-tool/guidance/reliability-safety

[8] Australian Government, National AI Centre. [n. d.]. Essential AI Practices. Australian Government. https://www.ai.gov.au/staying-safe-and-responsible/essential-ai-practices

[9] Australian Government, National AI Centre. 2026. Guidance for AI Adoption: Foundations. Australian Government. Published 5 May 2026. https://www.ai.gov.au/staying-safe-and-responsible/essential-ai-practices/guidance-ai-adoption-foundations

[10] Stephen J Ball, Meg Maguire, and Annette Braun. 2011. *How schools do policy: Policy enactments in secondary schools*. Routledge.

[11] Eric PS Baumer. 2015. Reflective informatics: conceptual dimensions for designing technologies of reflection. In *Proceedings of the 33rd annual ACM conference on human factors in computing systems*. 585–594.

[12] Juana Catalina Becerra Sandoval and Felicia S Jing. 2025. Rethinking AI Safety: Provocations from the History of Community-based Practices of Road and Driver Safety. In *Proceedings of the 2025 ACM Conference on Fairness, Accountability, and Transparency*. 964–974.

[13] Emily M Bender, Timnit Gebru, Angelina McMillan-Major, and Shmargaret Shmitchell. 2021. On the dangers of stochastic parrots: Can language models be too big?. In *Proceedings of the 2021 ACM conference on fairness, accountability, and transparency*. 610–623.

[14] Marit Bentvelzen, Paweł W Woźniak, Pia SF Herbes, Evropi Stefanidi, and Jasmin Niess. 2022. Revisiting reflection in hci: Four design resources for technologies that support reflection. *Proceedings of the ACM on interactive, mobile, wearable and ubiquitous technologies* 6, 1 (2022), 1–27.

[15] Charlotte Blease, Josefin Hagström, Carolina Garcia Sanchez, Anna Kharko, Brian McMillan, Jens Gaab, Emma Brulin, Cosima Locher, Maria Hägglund, Sara Riggare, et al. 2025. General practitioners' adoption of generative artificial intelligence in clinical practice in the UK: An updated online survey. *Digital Health* 11 (2025), 20552076251394287.

[16] Annette Braun, Meg Maguire, and Stephen J Ball. 2010. Policy enactments in the UK secondary school: Examining policy, practice and school positioning. *Journal of education policy* 25, 4 (2010), 547–560.

[17] Virginia Braun and Victoria Clarke. 2012. *Thematic analysis*. American Psychological Association.

[18] Virginia Braun and Victoria Clarke. 2019. Reflecting on reflexive thematic analysis. *Qualitative research in sport, exercise and health* 11, 4 (2019), 589–597.

[19] Zana Buçinca, Maja Barbara Malaya, and Krzysztof Z Gajos. 2021. To trust or to think: cognitive forcing functions can reduce overreliance on AI in AI-assisted decision-making. *Proceedings of the ACM on Human-computer Interaction* 5, CSCW1 (2021), 1–21.

[20] Carrie J Cai, Samantha Winter, David Steiner, Lauren Wilcox, and Michael Terry. 2019. " Hello AI": uncovering the onboarding needs of medical practitioners for human-AI collaborative decision-making. *Proceedings of the ACM on Human-computer Interaction* 3, CSCW (2019), 1–24.

[21] Runze Cai, Nuwan Janaka, Yang Chen, Lucia Wang, Shengdong Zhao, and Can Liu. 2024. PANDALens: Towards AI-assisted in-context writing on OHMD during travels. In *Proceedings of the 2024 CHI Conference on Human Factors in Computing Systems*. 1–24.

[22] Carnegie Mellon University. [n. d.]. Responsible Use of Generative AI in the Classroom. Eberly Center for Teaching Excellence and Educational Innovation. https://www.cmu.edu/teaching/technology/aitools/use/index.html

[23] John M Carroll, Jeongwon Jo, Jiyoon Kim, Ya-Fang Lin, and Ester Chen. 2026. What Happened to Scenario-Based Design in HCI?: A Scoping Review. In *Proceedings of the 2026 CHI Conference on Human Factors in Computing Systems*. 1–29.

[24] Valerie Chen, Alan Zhu, Sebastian Zhao, Hussein Mozannar, David Sontag, and Ameet Talwalkar. 2025. Need help? designing proactive ai assistants for programming. In *Proceedings of the 2025 CHI Conference on Human Factors in Computing Systems*. 1–18.

[25] Katy Cleece, Bethany Cheneler, Joanna Harrison, and James Edward Hill. 2025. Relational practice, a critical component for successful social work. *Cogent social sciences* 11, 1 (2025), 2492402.

[26] Kyle Crichton, Abhiram Reddy, and Jessica Ji. 2026. *Operationalizing AI Guidance: A Reference Guide for Translating High-Level Goals into Practical Implementation*. Technical Report. Center for Security and Emerging Technology, Georgetown University. https://cset.georgetown.edu/publication/operationalizing-ai-guidance-a-reference-guide-for-translating-high-level-goals-into-practical-implementation/

[27] Kyle Crichton, Abhiram Reddy, Jessica Ji, Ali Crawford, Mia Hoffmann, Colin Shea-Blymyer, and John Bansemer. 2025. Harmonizing AI Guidance: Distilling Voluntary Standards and Best Practices into a Unified Framework. *Center for Security and Emerging Technology, https://cset. georgetown. edu/publication/harmonizing-ai-guidance-distilling-voluntary-standards-and-best-practices-into-a-unified-framework* (2025).

[28] Aayushi Dangol, Smriti Kotiyal, Robert Wolfe, Alex J Bowers, Antonio Vigil, Jason Yip, Julie A Kientz, Suleman Shahid, Tom Yeh, Vincent Cho, et al. 2026. Relief or displacement? How teachers are negotiating generative AI's role in their professional practice. In *Proceedings of the 2026 CHI Conference on Human Factors in Computing Systems*. 1–21.

[29] Ankolika De, Gabriel Lima, and Yixin Zou. 2026. What is Safety? Corporate Discourse, Power, and the Politics of Generative AI Safety. In *Proceedings of the 2026 CHI Conference on Human Factors in Computing Systems*. 1–18.

[30] Gweneth Hartrick Doane and Colleen Varcoe. 2007. Relational practice and nursing obligations. *Advances in nursing science* 30, 3 (2007), 192–205.

[31] Joseph Dunne. 2005. An intricate fabric: Understanding the rationality of practice. *Pedagogy, Culture & Society* 13, 3 (2005), 367–390.

[32] Jasper S. Faber, Charlotte C. Poot, Tessa Dekkers, Natalia Romero Herrera, Niels H. Chavannes, Eline Meijer, and V. T. Visch. 2023. Developing a Digital Medication Adherence Intervention for and With Patients With Asthma and Low Health Literacy: Protocol for a Participatory Design Approach. *JMIR Formative Research* 7 (2023), e35112. doi:10.2196/35112

[33] Paul Festor, Myura Nagendran, Anthony C Gordon, Aldo A Faisal, and Matthieu Komorowski. 2025. Safety of human-AI cooperative decision-making within intensive care: A physical simulation study. *PLOS Digital Health* 4, 2 (2025), e0000726.

[34] Karen L Fortuna, Amanda L Myers, Danielle Walsh, Robert Walker, George Mois, and Jessica M Brooks. 2020. Strategies to increase peer support specialists' capacity to use digital technology in the era of COVID-19: Pre-post study. *JMIR mental health* 7, 7 (2020), e20429.

[35] Cheryl A Gagne, Wanda L Finch, Keris J Myrick, and Livia M Davis. 2018. Peer workers in the behavioral and integrated health workforce: opportunities and future directions. *American journal of preventive medicine* 54, 6 (2018), S258–S266.

[36] Deep Ganguli, Liane Lovitt, Jackson Kernion, Amanda Askell, Yuntao Bai, Saurav Kadavath, Ben Mann, Ethan Perez, Nicholas Schiefer, Kamal Ndousse, et al. 2022. Red teaming language models to reduce harms: Methods, scaling behaviors, and lessons learned. *arXiv preprint arXiv:2209.07858* (2022).

[37] Jason B Gibbons, Sachini Bandara, Vivian Flanagan, Carlos Hardy, Marla Oros, and Brendan Saloner. 2025. Association between peer support services, treatment engagement and overdose risk among Kentucky Medicaid beneficiaries with opioid use disorder. *Drug and alcohol dependence* (2025), 112944.

[38] Google. 2024. Responsible Generative AI Toolkit. Google AI for Developers. Last updated July 18, 2024. https://ai.google.dev/responsible

[39] Google PAIR. 2019. People + AI Guidebook. People + AI Research, Google. Published May 8, 2019; updated May 18, 2021. https://pair.withgoogle.com/guidebook

[40] Rex Haigh and Nick Benefield. 2024. The development of a relational practice movement. In *Relationships and mental health: Relational experience in distress and recovery*. Springer, 259–282.

[41] Kimberly Horn, Ryan E Flinn, Angela Marie Hagaman, Kristyn Zajac, Lauren A Hoffman, Melissa N Poulsen, Camille C Cioffi, Ducel Jean-Berluche, Ethan Spana, Patrick F Hibbard, et al. 2025. Systematizing peer recovery support services for substance use disorder: a taxonomy for measuring recovery milestones. *Frontiers in Public Health* 13 (2025), 1529078.

[42] Jane Hsieh, Miranda Karger, Lucas Zagal, and Haiyi Zhu. 2023. Co-designing alternatives for the future of gig worker well-being: Navigating multi-stakeholder incentives and preferences. In *Proceedings of the 2023 ACM Designing Interactive Systems Conference*. 664–687.

[43] Zhuoqun Jiang, ShunYi Yeo, Dorien Herremans, and Simon Tangi Perrault. 2026. Scaffolded Vulnerability: Chatbot-Mediated Reciprocal Self-Disclosure and Need-Supportive Interaction in Couples. In *Proceedings of the 2026 CHI Conference on Human Factors in Computing Systems*. 1–39.

[44] Anna Kawakami, Amanda Coston, Haiyi Zhu, Hoda Heidari, and Kenneth Holstein. 2024. The situate AI Guidebook: Co-designing a toolkit to support multi-stakeholder, early-stage deliberations around public sector AI proposals. In *Proceedings of the 2024 CHI Conference on Human Factors in Computing Systems*. 1–22.

[45] Anna Kawakami, Luke Guerdan, Yanghuidi Cheng, Kate Glazko, Matthew Lee, Scott Carter, Nikos Arechiga, Haiyi Zhu, and Kenneth Holstein. 2023. Training towards critical use: Learning to situate ai predictions relative to human knowledge. In *Proceedings of The ACM Collective Intelligence Conference*. 63–78.

[46] Wm Matthew Kennedy and Daniel Vargas Campos. 2024. Vernacularizing taxonomies of harm is essential for operationalizing holistic AI safety. In *Proceedings of the AAAI/ACM Conference on AI, Ethics, and Society*, Vol. 7. 698–710.

[47] Jinyan Kuang, Amber L Pope, and Yixuan Zhang. 2026. Psychotherapists' trust, distrust, and generative AI practices in psychotherapy: Qualitative study. *Journal of Medical Internet Research* 28 (2026), e88932.

[48] Tzu-Sheng Kuo, Hong Shen, Jisoo Geum, Nev Jones, Jason I Hong, Haiyi Zhu, and Kenneth Holstein. 2023. Understanding frontline workers' and unhoused individuals' perspectives on ai used in homeless services. In *Proceedings of the 2023 CHI Conference on Human Factors in Computing Systems*. 1–17.

[49] Tin Lai, Yukun Shi, Zicong Du, Jiajie Wu, Ken Fu, Yichao Dou, and Ziqi Wang. 2023. Psy-llm: Scaling up global mental health psychological services with ai-based large language models. *arXiv preprint arXiv:2307.11991* (2023).

[50] Michelle S Lam, Omar Shaikh, Hallie Xu, Alice Guo, Diyi Yang, Jeffrey Heer, James A Landay, and Michael S Bernstein. 2026. Just-In-Time Objectives: A General Approach for Specialized AI Interactions. In *Proceedings of the 2026 CHI Conference on Human Factors in Computing Systems*. 1–26.

[51] Gary Lamph, Rebecca Nowland, Paul Boland, Jayn Pearson, Catriona Connell, Vanessa Jones, Ellie Wildbore, Danielle L Christian, Catherine Harris, Joanne Ramsden, et al. 2023. Relational practice in health, education, criminal justice, and social care: a scoping review. *Systematic Reviews* 12, 1 (2023), 194.

[52] Karim Lekadir, Alejandro F. Frangi, Antonio R. Porras, Ben Glocker, Celia Cintas, Curtis P. Langlotz, et al. 2025. FUTURE-AI: International Consensus Guideline for Trustworthy and Deployable Artificial Intelligence in Healthcare. *The BMJ* 388 (2025), e081554. doi:10.1136/bmj-2024-081554

[53] Alex Jiahong Lu. 2026. Confronting the discursive–material power of storyboards: Toward participatory storyboarding. In *Proceedings of the 2026 CHI Conference on Human Factors in Computing Systems*. 1–16.

[54] Michal Luria and Stuart Candy. 2022. Letters from the future: Exploring ethical dilemmas in the design of social agents. In *Proceedings of the 2022 CHI conference on human factors in computing systems*. 1–13.

[55] Shuai Ma, Ying Lei, Xinru Wang, Chengbo Zheng, Chuhan Shi, Ming Yin, and Xiaojuan Ma. 2023. Who should i trust: Ai or myself? leveraging human and ai correctness likelihood to promote appropriate trust in ai-assisted decision-making. In *Proceedings of the 2023 CHI Conference on Human Factors in Computing Systems*. 1–19.

[56] Michael A Madaio, Luke Stark, Jennifer Wortman Vaughan, and Hanna Wallach. 2020. Co-designing checklists to understand organizational challenges and opportunities around fairness in AI. In *Proceedings of the 2020 CHI conference on human factors in computing systems*. 1–14.

[57] Nora McDonald, Sarita Schoenebeck, and Andrea Forte. 2019. Reliability and inter-rater reliability in qualitative research: Norms and guidelines for CSCW and HCI practice. *Proceedings of the ACM on human-computer interaction* 3, CSCW (2019), 1–23.

[58] Shery Mead. 2004. Peer support: What makes it unique. *Unpublished manuscript* (2004).

[59] Shery Mead, David Hilton, and Laurie Curtis. 2001. Peer support: a theoretical perspective. *Psychiatric rehabilitation journal* 25, 2 (2001), 134.

[60] Shery Mead, Eri Kuno, and Sarah Knutson. 2013. Intentional peer support. *Vertex (Buenos Aires, Argentina)* 24, 112 (2013), 426–433.

[61] Microsoft. [n. d.]. Microsoft Copilot Studio Implementation Guidance. Microsoft Learn. https://learn.microsoft.com/en-us/microsoft-copilot-studio/guidance/overview

[62] Joanna Astrid Miler, Hannah Carver, Rebecca Foster, and Tessa Parkes. 2020. Provision of peer support at the intersection of homelessness and problem substance use services: a systematic 'state of the art'review. *BMC Public Health* 20, 1 (2020), 641.

[63] Margaret Mitchell, Simone Wu, Andrew Zaldivar, Parker Barnes, Lucy Vasserman, Ben Hutchinson, Elena Spitzer, Inioluwa Deborah Raji, and Timnit Gebru. 2019. Model Cards for Model Reporting. In *Proceedings of the Conference on Fairness, Accountability, and Transparency*. 220–229. doi:10.1145/3287560.3287596

[64] Neema Moraveji, Jason Li, Jiarong Ding, Patrick O'Kelley, and Suze Woolf. 2007. Comicboarding: using comics as proxies for participatory design with children. In *Proceedings of the SIGCHI conference on Human factors in computing systems*. 1371–1374.

[65] Hussein Mozannar, Arvind Satyanarayan, and David Sontag. 2022. Teaching humans when to defer to a classifier via exemplars. In *Proceedings of the aaai conference on artificial intelligence*, Vol. 36. 5323–5331.

[66] Andrea Nakakawa, Jenny Löyttynen, Daniela Sangiorgi, and Lena Marmstål Hammar. 2026. Translating person-centered care into homecare practice: challenges and innovations in implementing dementia policy in Sweden. *Frontiers in Dementia* 5 (2026), 1814577.

[67] National Alliance on Mental Illness. 2026. Workforce: Peer Support Workers. https://www.nami.org/advocacy-at-nami/policy-positions/improving-health/workforce-peer-support-workers/

[68] OECD. 2026. *OECD Due Diligence Guidance for Responsible AI*. Technical Report. OECD Publishing, Paris. doi:10.1787/41671712-en

[69] Office of the Australian Information Commissioner. 2024. Checklist: Privacy Considerations When Using Commercially Available AI Products. Australian Government. https://www.oaic.gov.au/__data/assets/pdf_file/0017/243071/Checklist-privacy-and-use-of-commercial-AI-products.pdf

[70] Office of the Australian Information Commissioner. 2024. Guidance on Privacy and the Use of Commercially Available AI Products. Australian Government. Updated 17 January 2025. https://www.oaic.gov.au/privacy/privacy-guidance-for-organisations-and-government-agencies/guidance-on-privacy-and-the-use-of-commercially-available-ai-products

[71] Onora O'Neill. 2007. Normativity and practical judgement. *Journal of Moral Philosophy* 4, 3 (2007), 393–405.

[72] OpenAI. [n. d.]. Responsible and Safe Use. OpenAI Academy. https://academy.openai.com/

[73] Laysha Ostrow and Stephania L Hayes. 2015. Leadership and characteristics of nonprofit mental health peer-run organizations nationwide. *Psychiatric Services* 66, 4 (2015), 421–425.

[74] Cindy Peng, Megan Chai, Gao Mo, Naveen Raman, Ningjing Tang, Shannon Pagdon, Margaret Swarbrick, Nev Jones, Fei Fang, and Hong Shen. 2026. Large Language Models in Peer-Run Community Behavioral Health Services: Understanding Peer Specialists and Service Users' Perspectives on Opportunities, Risks, and Mitigation Strategies. In *Proceedings of the 2026 CHI Conference on Human Factors in Computing Systems*. 1–24.

[75] Terry L Pertuit. 2026. Generative AI as a Relational Bridge in Couples Counseling: A Supplemental Approach. *The Family Journal* (2026), 10664807261448228.

[76] Melina Petsolari, Seray B Ibrahim, and Petr Slovak. 2024. Socio-technical imaginaries: Envisioning and understanding AI parenting supports through design fiction. In *Proceedings of the 2024 CHI Conference on Human Factors in Computing Systems*. 1–27.

[77] Madison Pickering, Helena Williams, Alison Gan, Weijia He, Hyojae Park, Francisco Piedrahita Velez, Michael L Littman, and Blase Ur. 2025. How humans communicate programming tasks in natural language and implications for end-user programming with LLMs. In *Proceedings of the 2025 CHI conference on human factors in computing systems*. 1–34.

[78] Yegan Pillay. 2025. Ethical decision-making guidelines for mental health clinicians in the artificial intelligence (AI) era. In *Healthcare*, Vol. 13. MDPI, 3057.

[79] Michael Prilla, Oliver Blunk, and Irene-Angelica Chounta. 2020. How Does Collaborative Reflection Unfold in Online Communities? An Analysis of Two Data Sets: Prilla Michael et al. *Computer Supported Cooperative Work (CSCW)* 29, 6 (2020), 697–741.

[80] Michael Prilla and Bettina Renner. 2014. Supporting collaborative reflection at work: A comparative case analysis. In *Proceedings of the 2014 ACM International Conference on Supporting Group Work*. 182–193.

[81] Afarin Rajaei and Katherine Hertlein. 2026. Conclusion to the Special Issue and Dialogic Reflections on Artificial Intelligence and Systemic Therapy: Competence, Ethics, and the Future of Relational Practice. *International Journal of*

Systemic Therapy (2026), 1–8.

[82] Inioluwa Deborah Raji, Andrew Smart, Rebecca N. White, Margaret Mitchell, Timnit Gebru, Ben Hutchinson, Jamila Smith-Loud, Daniel Theron, and Parker Barnes. 2020. Closing the AI Accountability Gap: Defining an End-to-End Framework for Internal Algorithmic Auditing. In *Proceedings of the 2020 Conference on Fairness, Accountability, and Transparency*. 33–44. doi:10.1145/3351095.3372873

[83] Elad Refoua, Karny Gigi, Inbar Levkovich, Dorit Hadar Shoval, Zohar Elyoseph, Iftach Tsafrir, Oori Pen, Tal Angert, and Yuval Haber. 2026. Generative AI as a de facto mental health provider: a policy brief and urgent call for regulation. *Frontiers in Digital Health* 8 (2026), 1708806.

[84] Tyler Reinmund, Lars Kunze, and Marina Denise Jirotka. 2026. Sociotechnical Challenge Modeling: A Design Method for Responsible AI in Healthcare and Social Welfare. In *Proceedings of the 2026 CHI Conference on Human Factors in Computing Systems*. 1–20.

[85] Sarah Rüller, Konstantin Aal, Peter Tolmie, Andrea Hartmann, Markus Rohde, and Volker Wulf. 2022. Speculative design as a collaborative practice: Ameliorating the consequences of illiteracy through digital touch. *ACM Transactions on Computer-Human Interaction* 29, 3 (2022), 1–58.

[86] Donald A. Schön. 1983. *The Reflective Practitioner: How Professionals Think in Action*. Basic Books, New York.

[87] Donald A. Schön. 1987. *Educating the Reflective Practitioner: Toward a New Design for Teaching and Learning in the Professions*. Jossey-Bass, San Francisco.

[88] Ashish Sharma, Inna W Lin, Adam S Miner, David C Atkins, and Tim Althoff. 2023. Human–AI collaboration enables more empathic conversations in text-based peer-to-peer mental health support. *Nature Machine Intelligence* 5, 1 (2023), 46–57.

[89] Renee Shelby, Shalaleh Rismani, Kathryn Henne, AJung Moon, Negar Rostamzadeh, Paul Nicholas, N'Mah Yilla-Akbari, Jess Gallegos, Andrew Smart, Emilio Garcia, et al. 2023. Sociotechnical harms of algorithmic systems: Scoping a taxonomy for harm reduction. In *Proceedings of the 2023 AAAI/ACM Conference on AI, Ethics, and Society*. 723–741.

[90] Siddharth Shukla, Prachet Balaji, Ilayda Ozsan McMillan, Marvyn R Arévalo Avalos, Harpreet Nagra, and Zara Dana. 2026. Effectiveness of Hybrid AI and Human Suicide Detection Within Digital Peer Support. *Journal of Clinical Medicine* 15, 5 (2026), 1929.

[91] Magenta B Simmons, Sharla Cartner, Roxxanne MacDonald, Sarah Whitson, Alan Bailey, and Ellie Brown. 2023. The effectiveness of peer support from a person with lived experience of mental health challenges for young people with anxiety and depression: a systematic review. *BMC psychiatry* 23, 1 (2023), 194.

[92] Jaemarie Solyst, Lauren Wilcox, and Michael Madaio. 2025. " The Conduit by which Change Happens": Processes, Barriers, and Support for Interpersonal Learning about Responsible AI. In *Proceedings of the 2025 CHI Conference on Human Factors in Computing Systems*. 1–15.

[93] Amy B Spagnolo, Carlos W Pratt, Yuane Jia, Matthew DeMasi, Rita Cronise, and Kenneth Gill. 2022. The competencies of telehealth peer support: Perceptions of peer support specialists and supervisors during the COVID-19 pandemic. *Community Mental Health Journal* 58, 7 (2022), 1386–1392.

[94] Cheryl D Stults, Meghan C Martinez, Nina K Szwerinski, Naveed Rabbani, and Veena G Jones. 2026. Interviews with clinicians about an ambient artificial intelligence documentation platform. *Journal of the American Medical Informatics Association* 33, 4 (2026), 821–826.

[95] Substance Abuse and Mental Health Services Administration. [n. d.]. Behavioral Health Career Descriptions. https://www.samhsa.gov/about/careers/behavioral-health-workforce/careers-by-state/descriptions

[96] Substance Abuse and Mental Health Services Administration. 2025. *Key Substance Use and Mental Health Indicators in the United States: Results from the 2024 National Survey on Drug Use and Health*. Technical Report. Center for Behavioral Health Statistics and Quality, Substance Abuse and Mental Health Services Administration.

[97] Margaret Swarbrick. 2007. Consumer-operated self-help centers. *Psychiatric Rehabilitation Journal* 31, 1 (2007), 76.

[98] Margaret Swarbrick and Nev Jones. 2026. Integrating Artificial Intelligence Into Peer Support. *Psychiatric Services* 77, 8 (2026), 762–766. doi:10.1176/appi.ps.20260080

[99] Elham Tabassi. 2023. *Artificial Intelligence Risk Management Framework (AI RMF 1.0)*. Technical Report NIST AI 100-1. National Institute of Standards and Technology. doi:10.6028/NIST.AI.100-1

[100] Sandra J Tanenbaum. 2011. Mental health consumer-operated services organizations in the US: citizenship as a core function and strategy for growth. *Health Care Analysis* 19, 2 (2011), 192–205.

[101] Ningjing Tang, Jiayin Zhi, Tzu-Sheng Kuo, Calla Kainaroi, Jeremy J Northup, Kenneth Holstein, Haiyi Zhu, Hoda Heidari, and Hong Shen. 2024. AI failure cards: Understanding and supporting grassroots efforts to mitigate AI failures in homeless services. In *Proceedings of the 2024 ACM Conference on Fairness, Accountability, and Transparency*. 713–732.

[102] David Toro-Tobon, Oscar Ponce Ponte, Victor M Montori, and Juan P Brito. 2026. Clinician in the loop: a flawed solution for AI oversight. *bmj* 393 (2026).

[103] University of Alabama at Birmingham. [n. d.]. Practical Guidance for Using AI. UAB Responsible AI. https://www.uab.edu/ai/responsible-ai/practical-guidance

[104] University of California, Santa Cruz. 2026. Guide: Use Artificial Intelligence (AI) Safely. Information Technology Services. https://its.ucsc.edu/get-support/it-guides/guide-use-artificial-intelligence-ai-safely/

[105] University of Edinburgh. [n. d.]. Using Generative AI in Your Work: Guidance for Staff. Information Services. https://information-services.ed.ac.uk/computing/comms-and-collab/elm/generative-ai-guidance-for-staff

[106] University of Illinois System. [n. d.]. AI Guidelines for Safe Use. Administrative Information Technology Services. https://www.aits.uillinois.edu/reference_library/a_i_guidelines

[107] U.S. General Services Administration. [n. d.]. AI Guide for Government. Technology Transformation Services, Centers of Excellence. https://coe.gsa.gov/coe/ai-guide-for-government/

[108] Laura Weidinger, Jonathan Uesato, Maribeth Rauh, Conor Griffin, Po-Sen Huang, John Mellor, Amelia Glaese, Myra Cheng, Borja Balle, Atoosa Kasirzadeh, et al. 2022. Taxonomy of risks posed by language models. In *Proceedings of the 2022 ACM conference on fairness, accountability, and transparency*. 214–229.

[109] Jessica Wolf. 2025. Changing the game? Increasing the impact of peer-run organizations. *Community Mental Health Journal* 61, 2 (2025), 234–243.

[110] Richmond Y Wong, Deirdre K Mulligan, Ellen Van Wyk, James Pierce, and John Chuang. 2017. Eliciting values reflections by engaging privacy futures using design workbooks. *Proceedings of the ACM on Human-Computer Interaction* 1, CSCW (2017), 1–26.

[111] Richmond Y Wong, Jason Caleb Valdez, Ashten Alexander, Ariel Chiang, Olivia Quesada, and James Pierce. 2023. Broadening privacy and surveillance: Eliciting interconnected values with a scenarios workbook on smart home cameras. In *Proceedings of the 2023 ACM Designing Interactive Systems Conference*. 1093–1113.

[112] World Health Organization. 2021. *Ethics and Governance of Artificial Intelligence for Health: WHO Guidance*. World Health Organization, Geneva. https://www.who.int/publications/i/item/9789240029200

[113] Muzhe Wu, Yanzhi Zhao, Shuyi Han, Michael Xieyang Liu, and Hong Shen. 2025. Ai lego: Scaffolding cross-functional collaboration in industrial responsible ai practices during early design stages. *arXiv preprint arXiv:2505.10300* (2025).

[114] Yiying Xiong, Yusen Zhai, and Russel Fulmer. 2026. Regulating Artificial Intelligence in Mental Health: A Content Analysis of Global Professional Ethical and Policy Guidelines. *Journal of Counseling & Development* (2026).

[115] Yaxuan Yin, Shamya Karumbaiah, Yujin Kim, and Devansh Saxena. 2026. When AI Breaks, Teachers Repair: Pedagogical Repair Work in Situated Classroom Practice. In *The 2026 ACM Conference on Fairness, Accountability, and Transparency*. 2206–2224.

[116] Angie Zhang and Min Kyung Lee. 2025. Knowledge Workers' Perspectives on AI Training for Responsible AI Use. In *Proceedings of the 2025 CHI Conference on Human Factors in Computing Systems*. 1–18.

A PeerAlly

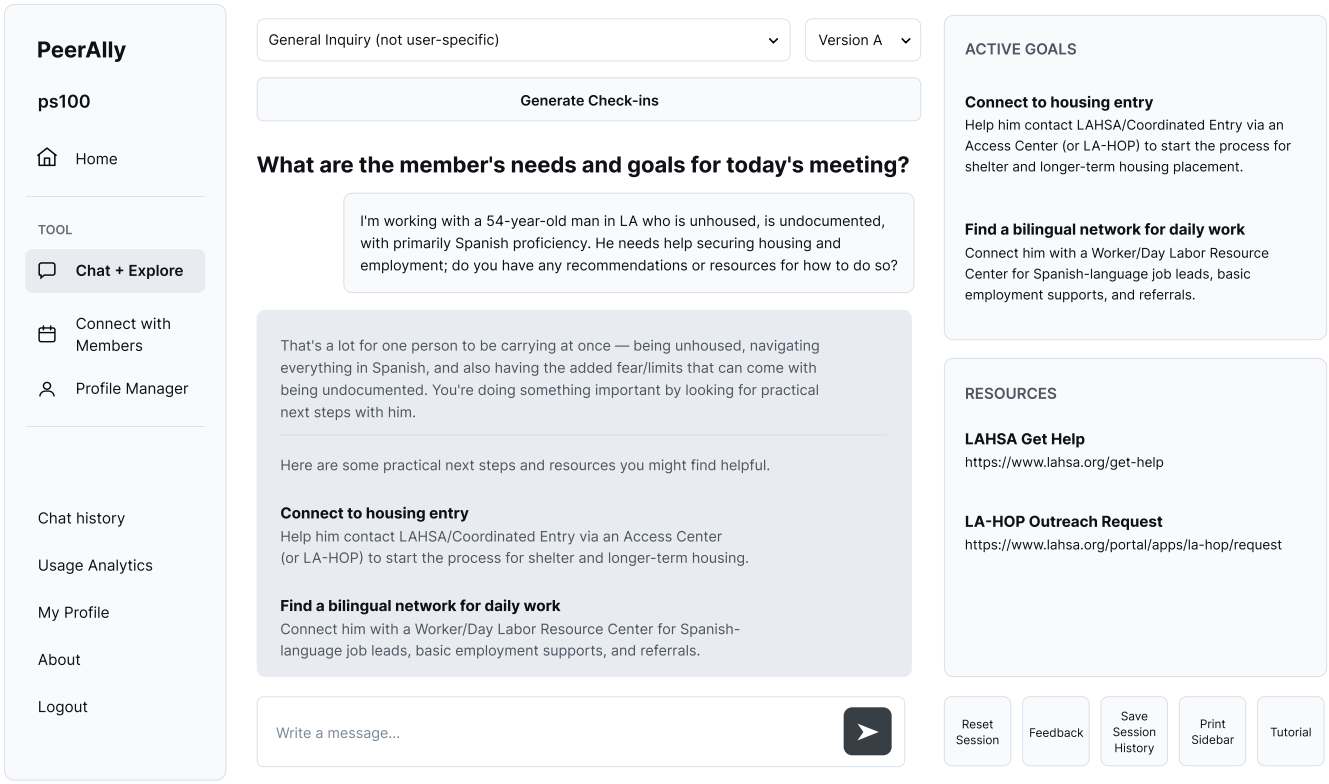


Figure 3: PeerAlly, an LLM-based assistant designed to support peer specialists in peer-support practice.

B AI Risk Scenario Cards

Figure 4 presents the five AI risk scenario cards used during the scenario-based elicitation activity. Participants viewed these cards while discussing what they found concerning, how they would respond, what contextual factors and competing considerations they would weigh, and what support would help them navigate each situation.

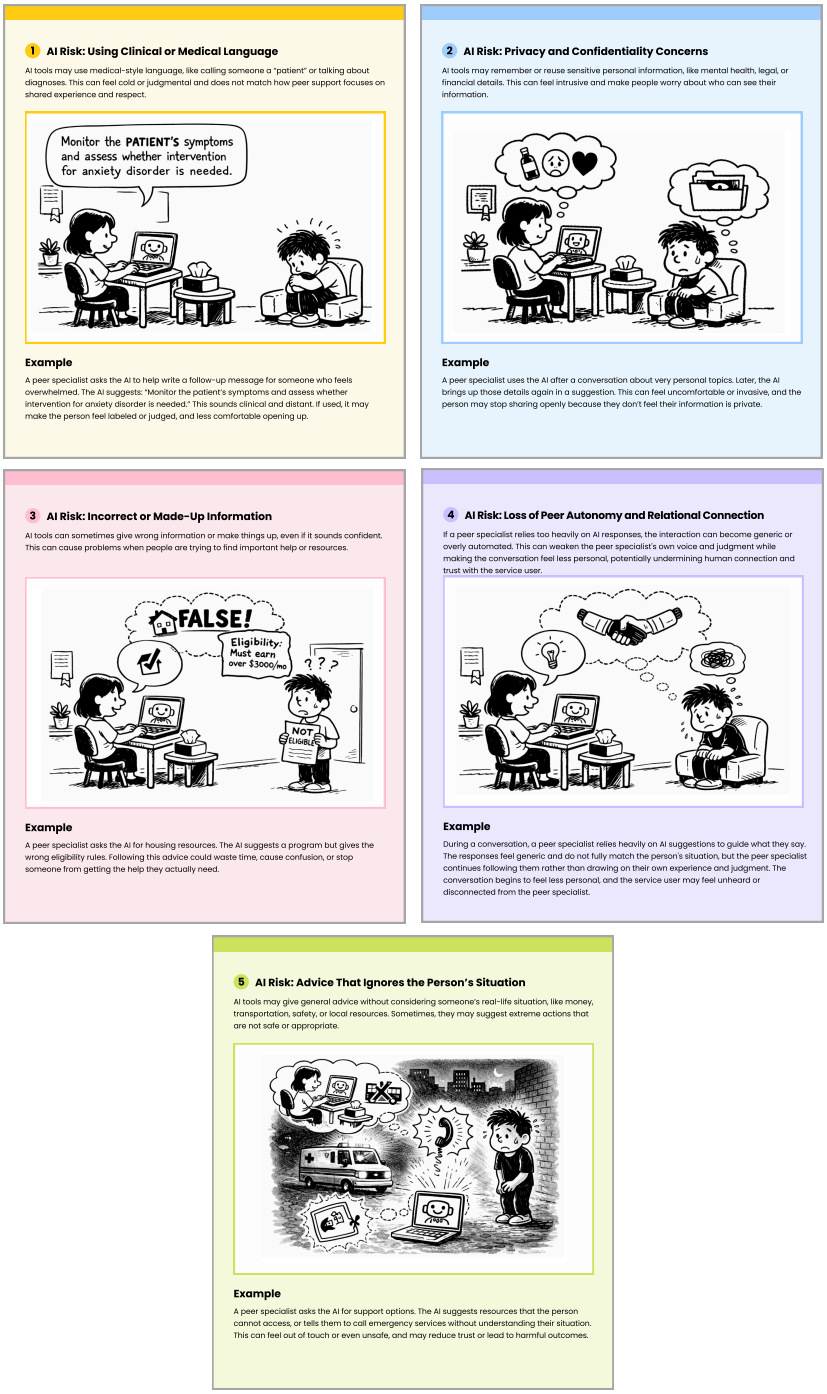


Figure 4: The five AI risk scenario cards used in the study.